

NEUROTRACK — 30-Day Monthly Clinical Summary

Physician Review Copy
Generated: 2026-09-05

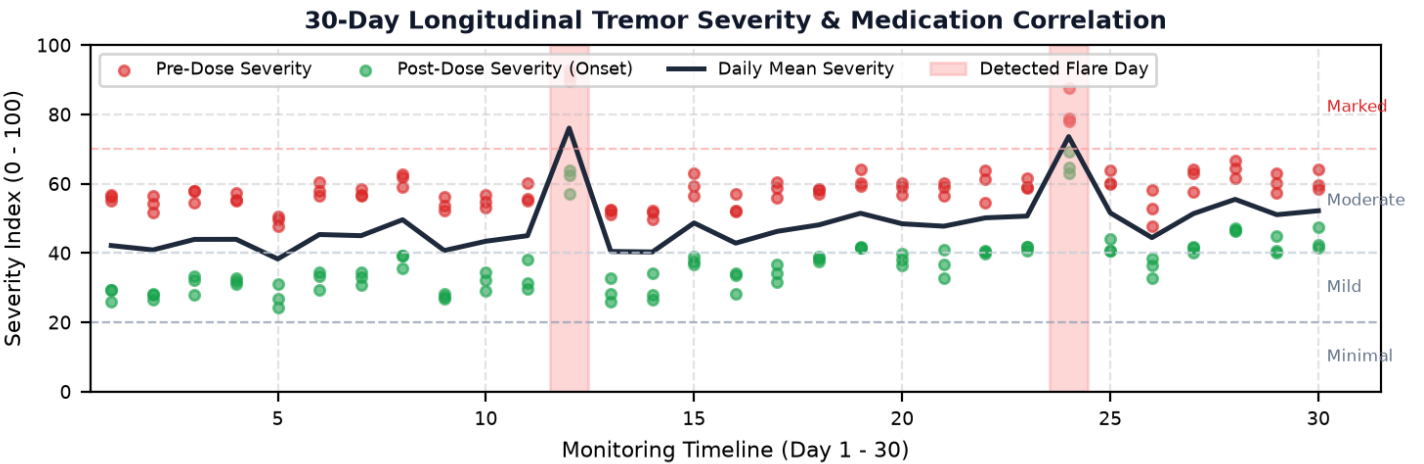
Patient ID:	LIVE_HW	Regimen:	Carbidopa/Levodopa 25/100 mg TID
Monitoring Window:	30 Calendar Days	Total Doses Logged:	90 scheduled doses
Telemetry Readings:	180 window assessments	Device Hardware:	Physical ESP32 MPU6050 (COM4)

Medication-Effectiveness Correlation Verdict: **REDUCED EFFECTIVENESS DETECTED** (Confidence: 78%)

Core Findings: Observed longitudinal wearing-off pattern. Average symptom drop decreased from 24.7 pts (Days 1-15) to 19.4 pts (Days 16-30). Trend slope: -0.33 pts/day. Recommend clinician review for potential motor fluctuation/wearing-off.

Dose Response Rate: 100.0% of doses showed significant symptom reduction (Mean drop: 22.0 severity points).

Longitudinal Tremor Severity Progression (30 Days)



Longitudinal Progression: Baseline Comparison (Week 1 vs. Week 4)

Monitoring Interval	Mean Pre-Dose Severity	Mean Post-Dose Severity	Net Medication Drop	Clinical Fluctuation Note
Week 1 (Days 1 - 7)	55.4 / 100	30.1 / 100	-25.2 pts	Robust therapeutic response; predictable ON phase window
Week 4 (Days 24 - 30)	63.2 / 100	45.2 / 100	-18.0 pts	Contracted therapeutic window; emerging wearing-off observed

Flagged Acute Flare Incidents (Non-Medication Spikes)

Calendar Day	Observed Day Severity	Baseline Elevation	Clinical Correlation Guidance
Day 12	76.0 / 100	+27.7 pts above mean	Investigate external factors (sleep disruption, infection, acute stress, physical fatigue)
Day 24	73.5 / 100	+25.3 pts above mean	Investigate external factors (sleep disruption, infection, acute stress, physical fatigue)

METHODOLOGY DISCLOSURE: In accordance with clinical trial protocol benchmarks, this 30-day longitudinal record is a simulated multi-session timeline constructed by modeling diurnal variance, pharmacokinetics, and wearing-off kinetics based on standardized single-session clinical IMU recordings.

CLINICAL DECISION-SUPPORT NOTICE: NeuroTrack is an investigational monitoring aid, NOT a diagnostic instrument or autonomous treatment recommendation tool. All longitudinal trends and medication response metrics are correlational patterns derived from simulated 30-day sensor telemetry. This report is intended exclusively to assist a licensed neurologist or attending physician in evaluating motor fluctuations. Dosage changes, prescription modifications, or treatment decisions must never be based solely on this automated summary.